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WOMEN'S HEALTH

Qualifying Exam Preparatory Course

COMPETENCIES COVERED IN THIS PRESENTATION...

**Providing Care:
Clinical Care**

**Providing Care:
Distribution**

**Knowledge
& Expertise**

**Communication
& Collaboration**

**Leadership &
Stewardship**

Professionalism

LEGEND

AE	Adverse Effect	NSAID	Non-Steroidal Anti-inflammatory Drug
AIDS	Acquired Immunodeficiency Syndrome	OTC	Over-The-Counter
BID	Bis In Die (Twice Daily)	PMDD	Premenstrual Dysphoric Disorder
BMI	Body Mass Index	PMN	Polymorphonucleocyte
BMD	Bone Mineral Density	PMS	Premenstrual Syndrome
BV	Bacterial Vaginosis	PO	Per Os (Oral)
Ca²⁺	Calcium	PRN	Pro Re Nata (As Needed)
CBT	Cognitive Behavioural Therapy	PV	Per Vaginal
COC	Combined Oral Contraceptive	Q	Quaque (Every)
CV	Cardiovascular	QTp	QT-interval Prolongation
DSM-5	Diagnostic and Statistic Manual of Mental Disorders 5 Ed	SSRI	Selective Serotonin Reuptake Inhibitor
GABA	Gamma-Aminobutyric Acid	STI	Sexually Transmitted Infection
GI	Gastrointestinal	UD	Ut Dictum (As Directed)
GnRH	Gonadotropin-Releasing Hormone	PMN	Polymorphonucleocyte
H	Hours	PMS	Premenstrual Syndrome
IM	Intramuscular	Q	Quaque (Every)
IUD	Intrauterine Device	TEN	Toxic Epidermal Necrolysis
Mg²⁺	Magnesium		

OBJECTIVES

For each women's health condition:

- 1 Explain the pathophysiology
- 2 Identify the clinical presentation, diagnostic method, and red flags
- 3 Understand goals of therapy
- 4 Manage with non-pharmacological and pharmacological measures
- 5 Identify monitoring parameters

PREMENSTRUAL SYNDROME

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PATHOPHYSIOLOGY

- **Premenstrual Syndrome (PMS):** broad term referring to physical, emotional, and behavioral symptoms occurring prior to the onset of menses and remitting with the onset of menses; incidence is 90%
- **Premenstrual Dysphoric Disorder (PMDD):** severe extension of PMS that can significantly impair daily functioning; incidence is 3-8%
- **Exact cause is unknown, possible causes:**

Changes in hormones

Signs and symptoms of PMS are affected by hormonal fluctuation and are no longer present with pregnancy and menopause

Neurotransmitters

Changes in estrogen and progesterone levels can change GABA and serotonin levels, contributing to PMS symptoms

Vitamins and minerals

PMS symptoms have been linked to magnesium, calcium, vitamin B and vitamin D deficiencies

Follicular phase

Begins shortly after the start of menses

Luteal phase

Begins after ovulation
Symptoms begin up to 7-10 days before menses

CLINICAL PRESENTATION

Affective (Behavioral) Symptoms

- Aggression, anger, irritability
- Anxiety or tension
- Diminished interest in activities
- Forgetfulness
- Hostility
- Mood swings
- Increased appetite or food cravings
- Lethargy
- Panic attacks
- Poor concentration
- Sad or depressed mood
- Sensitivity to rejection

Physical Symptoms

- Abdominal bloating
- Acne
- Appetite changes
- Breast pain or swelling
- Constipation
- Dizziness or vertigo
- Extreme sense of fatigue
- Headaches
- Hot flashes
- Muscle aches
- Nausea/vomiting
- Pelvic heaviness or pressure
- Weight gain

DIAGNOSIS

- **There is no currently accepted diagnostic test**
- *Differential diagnoses:* anemia, diabetes, thyroid disorders, chronic fatigue syndrome, endometriosis, polycystic ovaries, irritable bowel syndrome, adverse effects from oral contraceptives, perimenopause, substance use disorders, fibrocystic breast changes and various psychiatric disorders
- Onset of PMS can occur at any time after puberty with typical onset in the mid-20s
- Perimenopausal women can suffer from PMS, but it subsides after menopause

A diagnosis of PMS is made if symptoms:

1. Are present during the luteal phase of the menstrual cycle but absent during follicular phase
2. Reach their peak shortly before the beginning of menstruation and remit at the onset of menses or shortly thereafter
3. One to four symptoms that may be physical, behavioural, or affective in nature that interfere with daily functioning and interpersonal relationships

Other diagnostic confirmations: Monitoring of a woman's symptoms during at least 2 menstrual cycles with a daily symptom calendar to determine if woman's symptoms are cyclical and are confined to the luteal phase



Presence of ≥ 5 symptoms, with one being "affective", is more accurate to diagnose a woman with PMDD rather than PMS

RISK FACTORS

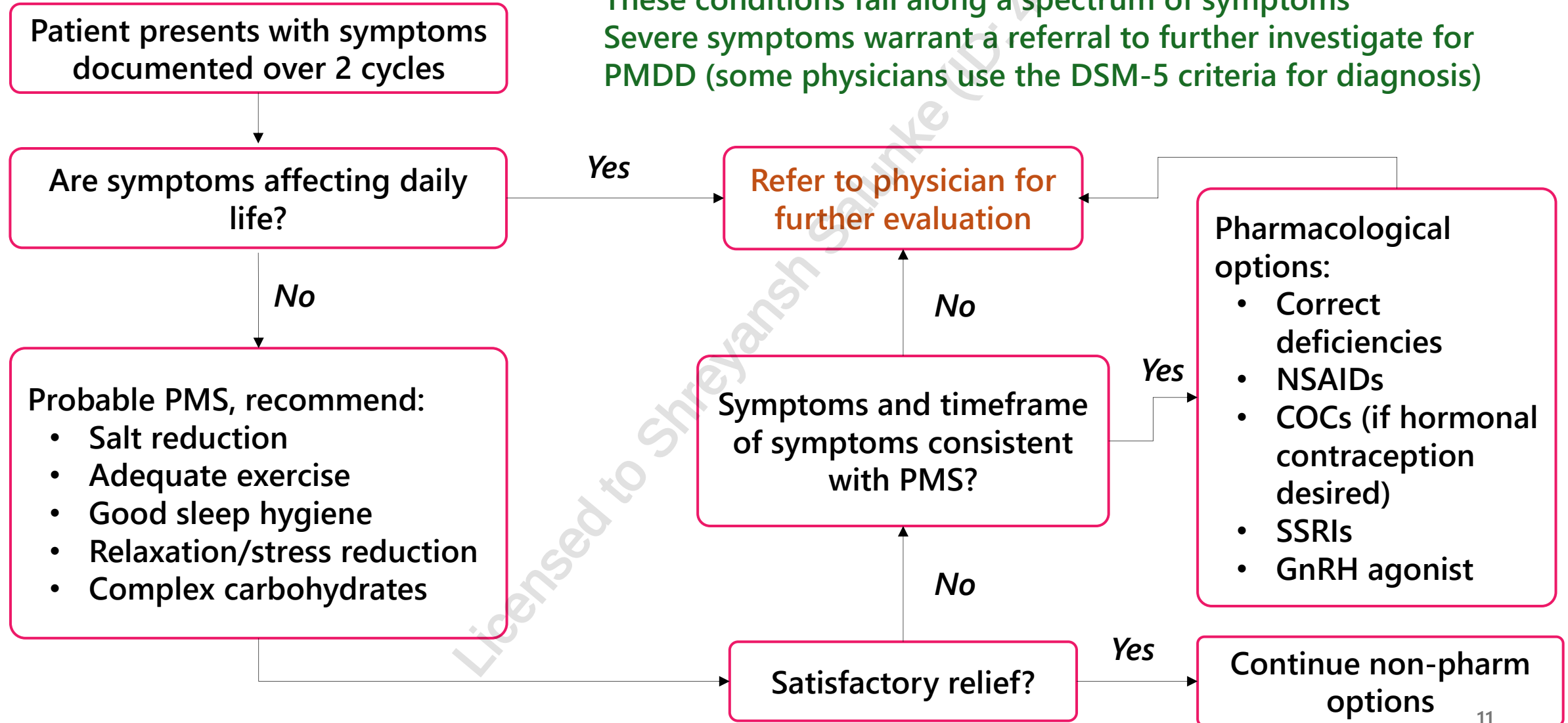
- **Age:** late 20s to early 40s
- High Body Mass Index (BMI)
- **Family history**
 - PMS is more present in patients whose mother experienced PMS
- **Nulliparity**
- **Mental health**
 - Current or history of mood or anxiety disorders
 - History of domestic violence, physical or emotional trauma, and substance misuse
- **Lifestyle factors**
 - **Diet high in salty foods** may induce bloating, fluid retention
 - **Alcohol and caffeine** worsen PMS symptoms
 - **Deficiencies** in certain minerals or vitamins, such as calcium, magnesium, manganese, vitamin D, B vitamins (e.g. thiamine, riboflavin), vitamin E and linoleic acid
 - **Stress** may worsen symptoms
 - **Exercise** may improve symptoms

GOALS OF THERAPY

- ✓ Relieve or eliminate symptoms
- ✓ Minimize functional impairment and improve quality of life
- ✓ Prevent symptom recurrence
- ✓ Minimise adverse effects associated with medications

MANAGEMENT

These conditions fall along a spectrum of symptoms
Severe symptoms warrant a referral to further investigate for PMDD (some physicians use the DSM-5 criteria for diagnosis)



NON-PHARMACOLOGICAL MEASURES

Lifestyle modification

- Cycle tracking
- Relaxation and stress reduction
- Sleep hygiene
- Moderate aerobic activity 3-4 times weekly to improve breast tenderness, fluid retention, stress, and depression
- Cognitive behavioural therapy (CBT)

Dietary modification

- Increase carbohydrate intake during the luteal phase (small frequent servings) to reduce appetite changes and cognitive symptoms
- Increase fluid intake (water) and restrict salt, caffeine, and alcohol intake in the luteal phase to alleviate fluid retention, weight gain, bloating, and breast swelling/tenderness

NATURAL HEALTH PRODUCTS

FYI

- Some patients may consider using natural health products e.g. calcium, chasteberry, primrose oil, magnesium, vitamin B6 (pyridoxine), or vitamin E
- Evidence for these agents is poor, and therefore are not routinely recommended
- Higher intake of vitamin B1 (thiamine) and vitamin B2 (riboflavin) from food sources have been associated with a reduced likelihood of developing PMS, compared with women who consume lower amounts (findings from one cohort study)

PHARMACOLOGICAL MEASURES

Class	Dosing	Comments
NSAIDs	Naproxen : 220 mg PO Q8-12H Ibuprofen : 200-400 mg Q6H PRN PO	- Relief for headache, breast pain, and myalgia - Start at onset of pain and used short term in the lowest effective dose
SSRIs	Citalopram , escitalopram , fluoxetine , paroxetine , or sertraline : during the luteal phase (days 14-start of menses) or continuously	Used short term for women who do not require contraception and have primarily mood symptoms
Combined oral contraceptives (COCs)	Ethinyl estradiol 20-30 mcg + drospirenone 3 mg PO as directed (UD)	- Use for women who have moderate-severe PMS and require contraception - Some women may prefer oral contraceptive monotherapy Higher risk of venous thromboembolic events with products that contain cyproterone or drospirenone
	<u>Others</u> : Ethinyl estradiol and various first, second and third generation progestogens (effectiveness inconsistent)	
GnRH agonists	Leuprolide : 3.75 mg IM monthly (most common) <u>Others</u> : goserelin , nafarelin , danazol	- Used for women with severe symptoms who did not tolerate/failed treatment with COCs or SSRIs AEs: ↓BMD, metabolic/CV effects, psychiatric events, QTp

MONITORING PARAMETERS

Symptoms

Encourage women to **self-monitor** their response to various treatment measures by **tracking** their **symptoms** throughout their cycle

Efficacy

Women with unrelenting or progressive symptoms despite drug therapy for 1–3 months (depending on drug and degree of response) should have treatment reassessed

- Assess or refer patients who have severe, debilitating symptoms with a strong affective or psychological component for assessment of possible PMDD

Safety

Adverse effects of drugs should be screened at appropriate intervals depending on therapy chosen

TAKEAWAY

- **Premenstrual syndrome (PMS)** refers to the physical and/or affective (emotional, behavioural) symptoms that occur prior to the onset of menses and improve once menstruation begins
- **Premenstrual dysphoric disorder (PMDD)** is a severe extension of PMS that may significantly impair daily functioning
- Non-pharmacologic options include: cycle tracking, relaxation techniques, stress reduction, sleep hygiene, aerobic activity, dietary modifications, and cognitive behavioural therapy (CBT)
- There is insufficient evidence to recommend natural health products (NHPs)
- Pharmacological options include: NSAIDs, SSRIs, COCs (if hormonal contraception is desired), and GnRH agonists (if other therapies fail)

CASE

AG is a 30-year-old female who comes to your pharmacy with complaints of fatigue, irritability, bloating, breast tenderness, acne breakouts and poor concentration for the past 2 days. She finds these symptoms frustrating because they are affecting her daily activities. Her last blood work revealed she had low vitamin D, so she now supplements with vitamin D 1000 units PO daily. She has a body mass index (BMI) of 24 kg/m² and she has no allergies to any medications.

1. What is the MOST important question to ask AG first?
 - a) Have you had trouble sleeping for the past 2 days?
 - b) How severe are your symptoms and are they debilitating?
 - c) Are you pregnant or is there a chance that you could be?
 - d) Are you breastfeeding?



CASE

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2. Which risk factor does AG have that would predispose her to PMS?

- a) Family history of PMS
- b) Female gender
- c) Vitamin D deficiency
- d) High BMI



VAGINITIS

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PATHOPHYSIOLOGY

Vaginal epithelium is composed of non-keratinized stratified squamous cells (premenopausal women) that is rich in glycogen

Estrogen induces the maturation of the epithelial cells and stimulates the proliferation of epithelial basal cells. It also increases the cells' ability to store glycogen

Lactobacilli use glycogen as a substrate for growth and to make lactic acid

The lactic acid produced by lactobacilli maintains an acidic vaginal environment (pH 3.5-5.5), which inhibits the growth of pathogenic organisms and supports normal vaginal health

Disruption of normal ecosystem/flora

Vaginitis

Definition

- Inflammation of the vagina that is commonly associated with pain, discharge, and pruritus

Common types

- Bacterial vaginosis
- Vulvovaginal candidiasis
- Trichomoniasis

Common causes

- Menstrual cycle
- Sexual activity
- Immunocompromised
- Contraceptives
- Antibiotics
- Hygiene products
- Pregnancy
- Tight fitting clothing

CLINICAL PRESENTATION AND DIAGNOSIS

		Vaginal Candidiasis	Trichomoniasis	Bacterial Vaginosis
Symptoms	Organism	<i>Candida albicans</i>	<i>Trichomonas vaginalis</i>	<i>Mycoplasma and Gardnerella vaginalis</i>
	Pruritus	+	+	-
	Odour	-	+	+ (fishy)
	Discharge	White, clumpy, curdy "cottage cheese"	Off-white or yellow, frothy	Grey or milky, thin, copious
	Inflammation	+	+	-
	Vaginal pH	<4.5 (normal)	>4.5	>4.5
	"Whiff" test	-	-	+
Findings	Specific	Budding yeast	Motile trichomonads	Gram-(-) curved bacilli and coccobacilli
	PMNs	++	+++	-
	Lactobacilli	+	-	-

GOALS OF THERAPY

- ✓ Relieve symptoms
- ✓ Eradicate the underlying cause
- ✓ Prevent recurrence and complications
- ✓ Prevent misdiagnosis and delayed treatment of another condition

NON-PHARMACOLOGICAL MEASURES

VAGINAL HYGIENE



- Wash perineal area daily with warm water and mild unscented soap
- May use lubricants (KY Jelly®, Astroglide®)
- Wear cotton underwear and pants



- Douching – no benefits and possible adverse outcomes
- Genital deodorants (can mask genital odour)
- Wear tight clothing and synthetic underwear

Complimentary Medicine

1. Black cohosh – has been used for vaginal dryness, **although evidence for efficacy is lacking** (contraindicated in pregnancy/breastfeeding)
2. Probiotics – **insufficient evidence**, although may recommend *L. acidophilus*

BACTERIAL VAGINOSIS

PHARMACOLOGICAL MEASURES

DRUG	DOSE	SIDE EFFECTS	COMMENTS
Metronidazole <i>Preferred regimen</i>	500 mg PO BID x 7 days - OR - Topical 0.75%: 1 applicatorful (5 g) daily x 5 days	ORAL: GI upset, metallic taste, urethral burning, dark or reddish-brown discoloration of urine TOPICAL: Vaginal discharge, yeast infection, vulva/vaginal irritative symptoms, pain and discomfort	Disulfiram-like reaction may occur when consuming alcohol; avoid alcohol Topical treatment has similar cure rate as oral metronidazole
Clindamycin <i>Preferred regimen</i>	Topical 2%: 1 applicatorful (5 g) daily x 7 days	Vulvovaginitis	Delay treatment until completion of menstrual period Contains mineral oil— may decrease effectiveness of condoms and diaphragms
Clindamycin <i>Alternative regimen</i>	300 mg PO BID x 7 days	GI upset, <i>Clostridioides difficile</i> colitis, diarrhea	



Topical metronidazole and clindamycin equally effective (Grade A evidence)

BACTERIAL VAGINOSIS

CLINICAL PEARLS

- Do not need to treat sexual partners
- BV resolves spontaneously in 1/3 of women
- Treatment is only indicated in symptomatic patients
- Treatment is **unnecessary** in asymptomatic patients except if:
 - Undergoing intrauterine device (IUD) insertion, gynecologic surgery, therapeutic abortion, upper genital tract instrumentation or if pregnant and at high risk of preterm delivery
- **Pregnancy:** Untreated bacterial vaginosis may cause premature rupture of membranes, preterm labour, preterm birth and postpartum endometritis

TRICHOMONIASIS

PHARMACOLOGICAL MEASURES

Preferred treatment

- Metronidazole: 2 g PO single dose
OR
- Metronidazole: 500 mg PO BID x 7 days

Alternative treatment

- None

Clinical Pearls

- Treat all patients
- Treat sexual partners irrespective of symptoms or test results
- Can resume sexual activity once both partners finish course and are asymptomatic
- No follow-up is necessary unless symptoms do not resolve or recur
- Intravaginal treatment is ineffective
- Pregnancy: Untreated trichomoniasis may lead to premature rupture of membranes, preterm delivery and low birth weight

UNCOMPLICATED VS COMPLICATED VAGINAL CANDIDIASIS

UNCOMPLICATED

- Criteria for uncomplicated infection:
 1. ≤ 3 episodes/year
 2. Mild symptoms
 3. Non-pregnant
 4. Probable pathogen: *C. albicans*
- Do not treat sexual partners
- Self-treat unless:
 1. First infection
 2. Age <12 years old (prepuberty)
 3. Fever and pelvic pain
 4. Less than 2 months since last infection

COMPLICATED

- Criteria for complicated infection:
 1. ≥ 4 episodes/year
 2. Severe symptoms
 3. Pregnancy, diabetes, AIDS
 4. Non-albicans species
- Do not treat sexual partners

UNCOMPLICATED VAGINAL CANDIDIASIS

PHARMACOLOGICAL MEASURES

DRUG	DOSE	SIDE EFFECTS	COMMENTS
Fluconazole (Diflucan One®) <i>Preferred regimen</i>	150 mg PO single dose	Headache, nausea, abdominal pain, diarrhea, dyspepsia, dizziness	Avoid during pregnancy where possible Do not use in <12 years old
Clotrimazole (Canesten®) <i>Preferred regimen</i>	- Vaginal tablets: 200 mg daily x 3 days or 500 mg once only - OR - Vaginal cream: 1% daily x 7 days or 2% daily x 3 days or 10% once only	Local hypersensitivity	Also available in combination with external cream Menstruation is not an indication to stop treatment Safe for use in pregnancy
Miconazole (Monistat®) <i>Preferred regimen</i>	- Vaginal cream: 2% daily x 7 days or 4% daily x 3 days - OR - Miconazole vaginal ovules: 100 mg daily x 7 days or 400 mg daily x 3 days or 1200 mg once only	Local hypersensitivity	Ovules contain hydrogenated vegetable oil and mineral oil, which may decrease effectiveness of condoms or diaphragms Safe for use in pregnancy
Terconazole <i>Preferred regimen</i>	Terconazole vaginal cream: 0.4% daily 1 applicator (5 g) x 7 days	Local hypersensitivity	Health Canada safety warning: Anaphylaxis and toxic epidermal necrolysis (TEN) - RARE

COMPLICATED VAGINAL CANDIDIASIS

PHARMACOLOGICAL MEASURES

Preferred treatment

Fluconazole: 150 mg PO Q72H X 3 doses, then weekly for 6 months

Alternative treatment

- **Clotrimazole:** Treat for 10-14 days (topically, as for uncomplicated vaginal candidiasis) then maintain with 500 mg vaginal tablets once monthly for 6 months

Non-albicans infection treatment

- **Boric acid:** 300-600 mg gelatin capsules PV daily x 14 days, then maintain with 300 mg capsules x 5 days/month beginning the first day of the menstrual cycle for 6 months
- Side effects: Local irritation, vaginal burning; more pronounced with higher dose
- **Caution patients that boric acid is toxic if ingested orally and may be fatal**
- **May be irritating to sexual partners exposed to boric acid**
- **Contraindicated** in pregnancy

MONITORING PARAMETERS

Condition	Signs and Symptoms	Monitoring	Endpoint of therapy
Vulvovaginal Candidiasis	- Vulvovaginal itching and burning, dyspareunia, discharge - Increased irritation (may indicate adverse reaction to product or inappropriate therapy)	- Patient: daily for 7–10 days - Health-care provider: after 7 days or next pharmacy visit	- Eradication of symptoms - Avoid side effects from regimen
Trichomoniasis	- Discharge - Pruritus - Odor	- Monitor for adverse events from treatment, symptoms improve in 2-3 days - Follow up after 7 days to ensure symptom resolution	- Eradication of symptoms - Educate on partner treatment
Bacterial Vaginosis	- Fishy odor - Discharge	- Signs of adverse event with antibiotics - Symptoms should improve in 2-3 days - Follow-up after 7 days to see resolution of symptoms	Eradication of symptoms



Refer if no improvement after 7 days

TAKEAWAY

- **Bacterial vaginosis:** metronidazole (PO or topical) or clindamycin (topical) are preferred
 - Clindamycin PO is an alternative regimen
 - Treatment is indicated in symptomatic patients and select asymptomatic patients
 - Sexual partners do not require treatment
- **Trichomoniasis (STI):** metronidazole PO (intravaginal treatment is ineffective)
 - Treat all sexual partners; resume sexual activity once therapeutic course is complete and when asymptomatic
- **Uncomplicated vaginal candidiasis:** fluconazole PO or topical agents (clotrimazole, miconazole, terconazole) are preferred
 - Results are not achieved more quickly than longer-course treatment (e.g. 7 days)
 - Sexual partners do not require treatment
- **Complicated vaginal candidiasis:** fluconazole PO is preferred
 - Alternatives: clotrimazole topical then PV, or boric acid PV
 - Typical treatment duration (for all agents): 6 months
 - Sexual partners do not require treatment
- Intravaginal products are usually administered at bedtime to increase contact time with vaginal tissue and should be continued through menses if it commences during therapy

CASE

QW is a 55-year-old female who comes to your pharmacy for advice on the bothersome symptoms she's been having for the last 3 days. She is experiencing white, clumpy discharge with some itching in her vaginal area. She is sexually active with her partner on a regular basis. She wants your advice on how to manage her symptoms with something over-the-counter (OTC) that does not involve swallowing tablets, and with a short treatment duration. She has hypertension and takes ramipril 5 mg PO once daily. She is allergic to ibuprofen (experiences a rash and stomach upset).

1. What is the most important question to ask QW first?
 - a) Are you taking any probiotics?
 - b) Is this the first time you are experiencing these symptoms?
 - c) Is your partner aware that you are having these symptoms?
 - d) What time of day do these symptoms occur?



CASE

QW is a 55-year-old female who comes to your pharmacy for advice on the bothersome symptoms she's been having for the last 3 days. She is experiencing white, clumpy discharge with some itching in her vaginal area. She is sexually active with her partner on a regular basis. She wants your advice on how to manage her symptoms with something over-the-counter (OTC) that does not involve swallowing tablets, and with a short treatment duration. She has hypertension and takes ramipril 5 mg PO once daily. She is allergic to ibuprofen (experiences a rash and stomach upset).

2. Upon further assessment, QW is deemed to have uncomplicated vaginal candidiasis. Which pharmacological option is best suited to her needs?
- a) Fluconazole 150 mg PO once
 - b) Clotrimazole 1% PV for 6 days
 - c) Miconazole 4% PV for 3 days
 - d) Clotrimazole 10% PV for 1 day



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CHANGE LOG

April 2025

- Minor formatting edits throughout

August 2025

- Formatting and grammar changes throughout
- Slide 3: Updated legend
- Slide 7: Updated clinical presentation, removed note on diagnosis
- Slide 8: Updated diagnostic information including note on PMDD, added IBS and substance use disorders to differential diagnosis
- Slide 9: Removed note on twins
- Slide 10: Updated goals of therapy
- Slide 11: Updated algorithm
- Slide 13: Added FYI to slide
- Slide 14: Added AE of GnRH agonists
- Slide 20: Removed FYI
- Slide 21: Updated goals of therapy
- Slide 23: Removed note recommending genital wash and associated brands
- Slide 26: Modified complicated criteria point 4 to 'non-albicans species'
- Slide 27: Updated topical clotrimazole doses to align with SOGC guidelines
- Slide 30: Updated takeaway
- Slides 31, 32: Added nature of allergy to ibuprofen in case
- Slide 33: Updated references

November 2025

- Slide 17: Updated wording of option B

March 2026

- Slide 2: Updated NAPRA competencies
- Slide 18: Changed answer choice A